

Micro Set 4. Positive Externalities & Merit Goods — Singapore Healthcare and Education

Instructions. This booklet contains a single case study on positive externalities and merit goods, followed by six short-answer questions ranging from 2 to 6 marks.

Section Case Study. Singapore’s Subsidised Healthcare and University Education

Table 1.1: Singapore Healthcare and Education Indicators, 2019–2024

Indicator	2019	2020	2021	2022	2023	2024
Polyclinic Subsidised Visits (millions)	5.2	4.8	5.0	5.6	6.1	6.4
Mean Subsidy per Polyclinic Visit, CHAS (S\$)	28	30	32	35	38	41
CHAS Cardholders (millions, year-end)	1.46	1.52	1.60	1.70	1.82	1.95
National Influenza Vaccination Rate (%)	25	38	42	39	33	35
Local University Cohort Participation Rate (%)	36.0	37.5	38.0	39.0	40.5	42.0
Mean Government Subsidy per University Student (S\$)	18000	19500	21000	22500	24000	25500
SkillsFuture Take-Up Rate (% of eligible adults)	17	24	27	32	36	40

Source: Adapted from Ministry of Health Singapore, Ministry of Education, and SkillsFuture Singapore Annual Report 2024.

Extract 1.1: Why Singapore subsidises healthcare and education

Singapore’s framework for healthcare and education subsidisation rests on a distinctive interpretation of merit-good theory. Both goods exhibit positive externalities in consumption: a vaccinated individual reduces the probability of infectious-disease transmission to others, and a more educated workforce raises aggregate productivity, innovation, and tax revenues. Both goods are also subject to information failure — consumers may systematically underestimate the long-term private benefits of preventive healthcare and post-secondary education at the moment they would need to commit resources.

The Community Health Assist Scheme (CHAS) provides means-tested subsidies for primary-care visits at participating GP and dental clinics. The mean subsidy per visit rose from S\$28 in 2019 to S\$41 in 2024, while the number of CHAS cardholders grew from 1.46 to 1.95 million over the period. “We are deliberately keeping the price of primary care low to encourage early consultation and prevent the progression of chronic conditions, which is far more costly downstream,” the Ministry of Health said. The expansion of CHAS contributed to a 23 per cent rise in polyclinic and CHAS-subsidised primary care visits between 2019 and 2024.

National influenza vaccination uptake illustrates the difficulty of correcting consumption-externality undersupply through subsidy alone. Despite vaccines being free or heavily subsidised, the national vaccination rate rose from 25 per cent in 2019 to a Covid-driven peak of 42 per cent in 2021, then fell back to 33–35 per cent. “Even when the financial barrier is removed, information

failure persists: many adults below 65 do not perceive themselves as at risk and discount the social benefit of reducing transmission,” a public-health official observed. The Health Promotion Board now combines subsidies with targeted information campaigns and workplace partnerships to address this.

In post-secondary education, the cohort participation rate at local universities rose from 36 per cent in 2019 to 42 per cent in 2024, supported by an increased mean government subsidy from S\$18000 to S\$25500 per student per year. The Ministry of Education’s framing is explicit: “Higher education generates positive externalities through skills accumulation, research output, and social cohesion that the private market would underprovide. We subsidise tuition to bring private consumption closer to the socially optimal level.”

SkillsFuture, launched in 2015, provides every Singaporean over 25 with periodic training credits to encourage continuous skills upgrading. Take-up rose from 17 per cent of eligible adults in 2019 to 40 per cent in 2024, accelerating particularly after the 2020 announcement of additional S\$500 top-ups during the pandemic. “SkillsFuture corrects two failures simultaneously,” an Institute of Policy Studies researcher noted. “The classic positive externality of training spillovers between firms, and the time-inconsistency problem where individuals systematically defer self-investment that would benefit them in the long run.”

Source: Adapted from Ministry of Health, Ministry of Education, SkillsFuture Singapore, and Institute of Policy Studies Working Paper Series 2024.

Question 1**[2 marks]**

Define “merit good” and identify, with reference to Extract 1.1, two characteristics that make university education a merit good.

Question 2**[2 marks]**

With reference to Table 1.1, calculate the percentage change in the number of polyclinic subsidised visits between 2019 and 2024. Show your working.

Question 3**[4 marks]**

Using evidence from Extract 1.1, explain why university education would be under-consumed in a free market.

Question 4**[4 marks]**

Explain how information failure, as described in Extract 1.1, weakens the effectiveness of subsidies on influenza vaccination in raising the national vaccination rate.

Question 5**[6 marks]**

With the aid of a fully labelled diagram, explain how a per-student subsidy on university tuition would correct the under-consumption of higher education in Singapore.

Question 6**[6 marks]**

Using evidence from Extract 1.1, explain why the Health Promotion Board combines vaccination subsidies with targeted information campaigns rather than relying on subsidies alone.

End of Set 4 — Questions